

Temporal Provider Utilization Policy

Synthetic Training Policy - Non-PHI

Defines how temporal utilization windows (30-day, 60-day, 90-day) and provider denial rates are used for claim-level risk assessment.

Policy ID	CLAIMOPS-POL-007
Effective Date	2026-04-25
Applies To	Synthetic claims, provider, diagnosis, and cost datasets in this repository
Source Type	Locally generated demo policy; not a real payer coverage policy

Policy Summary

- provider_claim_count_30d measures recent utilization — the number of claims filed by this provider in the trailing 30-day window.
- provider_claim_count_60d provides a medium-term utilization signal for trend detection.
- provider_claim_count_90d establishes a longer baseline. Deviation from this baseline flags pattern changes.
- provider_30d_denial_rate above 30% in the trailing 30 days triggers enhanced claim scrutiny.
- low_volume_provider_risk activates when a provider's total claim count falls below statistical reliability thresholds.

Rules

Rule ID	Requirement
TEMP-01	provider_claim_count_30d captures short-term utilization. Fewer than 3 claims in the trailing 30 days produces an unstable risk signal. Use 60d and 90d windows as supplementary context.
TEMP-02	provider_claim_count_60d provides medium-term trend detection. A sharp increase from 30d to 60d may indicate a practice-pattern change or seasonal variation. A sharp decrease may indicate reduced patient volume.
TEMP-03	provider_claim_count_90d establishes the long-term baseline. Claims from providers whose 30d count deviates more than 50% from their 90d average should be reviewed for utilization anomalies.
TEMP-04	provider_30d_denial_rate measures the proportion of denied claims in the trailing 30 days. A denial rate exceeding 30% is elevated and triggers enhanced review for every subsequent claim from that provider until the rate normalizes.
TEMP-05	low_volume_provider_risk is set to 1 when a provider has fewer than 10 total claims in the reference period. Low-volume providers have statistically unreliable denial-rate estimates. Their claims are assessed primarily on dx-px compatibility and cost signals rather than provider history.

Remediation Guidance

- For low-volume providers, rely on coding and cost signals rather than historical denial patterns.
- For providers with elevated 30d denial rates, flag all pending claims for pre-submission review.

- For providers with unusual temporal patterns, request a utilization summary before adjudication.
- Do not include provider identifier or claim counts in LLM prompts. Use temporal descriptors only.

RAG Safety Note

This document contains only synthetic policy text. It intentionally avoids patient names, patient identifiers, service dates, and real billed amounts. Use policy_id, rule_id, denial reason, diagnosis category, and procedure status as retrieval metadata.